

REGULATORY & COMPLIANCE

Telepsychiatry & Controlled-Substance Prescribing Compliance Kit

Where the DEA telemedicine rules stand in September 2026, a pre-prescribing checklist, a treatment agreement for controlled medication, and a plan for the January 2027 deadline.

AUDIENCE
Psychiatrists · Prescribers

FORMAT
Status + checklists + forms

VERSION
1.0 · Sept 2026

TIME TO ADAPT
1 hour

WHAT IT IS

A plain-English status report on the federal rules for prescribing controlled medication by telemedicine, a checklist for each prescribing decision, a patient treatment agreement, and a contingency plan in case the temporary rules lapse.

USE IT WHEN

You prescribe Schedule II to V medication by video or phone, share patients with a prescriber, or need to plan for January 1, 2027.

CUSTOMIZE FIRST

Your DEA and state registrations, your states of practice, your EHR's e-prescribing setup, and your practice's policy on stimulants, benzodiazepines and opioid use disorder medication.

THE RULES ARE TEMPORARY AND MAY CHANGE WITHOUT WARNING

Everything in Part A reflects sources reviewed in September 2026. The federal flexibilities are extended by rule, not permanent law. Check the DEA Diversion Control Division and your state board before relying on any statement here.

Part A: Where things stand

Question	Status as of September 2026
What is the baseline rule?	Under the Ryan Haight Act, a practitioner may prescribe a controlled substance by remote (telemedicine) means only after at least one in-person medical evaluation, unless a statutory exception applies.
What is the current flexibility?	DEA and HHS issued a fourth temporary extension of the pandemic-era flexibilities. From January 1, 2026 through December 31, 2026, a DEA-registered practitioner may prescribe Schedule II to V controlled medication by telemedicine without a prior in-person evaluation, if the required conditions are met.
What conditions apply?	The prescription must be for a legitimate medical purpose in the usual course of professional practice, by a practitioner acting within federal law and the law of the state involved. The extension does not remove any of those requirements.
What happens on January 1, 2027?	Unless DEA extends the flexibilities again or a final rule takes effect, the pre-pandemic in-person requirement returns. As of the sources we reviewed, the proposed Special Registration for Telemedicine (published January 2025) had not been finalized.
Are there separate rules for buprenorphine?	Yes. Two final rules published in January 2025 address buprenorphine treatment via telemedicine and continuity of care for VA patients. They carry their own conditions. Read the current text before relying on them.
Do state rules add anything?	Yes, often. Many states have their own telemedicine, controlled-substance and PDMP rules and some are stricter than the federal baseline.

WHERE TO CHECK EACH MONTH DEA Diversion Control Division telemedicine page (deadiversion.usdoj.gov), the Federal Register, HHS Telehealth.HHS.gov, your state medical board and your professional society. Put a monthly reminder in your compliance calendar.

Part B: Pre-prescribing checklist (per new patient)

Complete before the first controlled-substance prescription, and update at each renewal cycle. Keep the answers in the record.

- ☐ **Identity.** Verified the patient's identity by a method appropriate to your practice (photo ID on camera, matched records, or another documented method).
- ☐ **Location.** Recorded the patient's physical location and address at the time of the visit.
- ☐ **License.** Hold a license or other legal authority in the patient's state at the time of the visit.
- ☐ **Registration.** Confirmed that my DEA registration and any required state controlled-substance registration cover the state where the patient is located. Requirements differ and are unsettled. Confirm with DEA and the state before prescribing.
- ☐ **Modality.** Used a real-time audio-video visit unless an audio-only exception applies to this medication and circumstance.
- ☐ **Evaluation.** Performed an evaluation adequate for a legitimate medical purpose, including psychiatric history, substance use history, medical history, current medication and other prescribers.

- ☐ **PDMP.** Reviewed the patient's prescription monitoring program record, and the records of neighboring states where required or prudent.
- ☐ **Alternatives and risk.** Considered non-controlled options and documented why the controlled medication is appropriate. Reviewed risks, including misuse, dependence and interactions.
- ☐ **Consent and agreement.** Obtained informed consent for telemedicine and, where I use one, a signed treatment agreement (Part D).
- ☐ **E-prescribing.** Used an EPCS system with the identity proofing and two-factor authentication DEA requires for electronic prescribing of controlled substances.
- ☐ **Pharmacy.** Confirmed the pharmacy and the medication are available where the patient is located.
- ☐ **Follow-up.** Set the follow-up interval, monitoring plan (for example, urine drug screening or labs where indicated) and a plan for in-person care if needed.

SPLIT TREATMENT If a psychologist or therapist shares the patient, agree who monitors adherence and what triggers a same-day call. A signed release makes that possible.

Part C: Documentation elements for a controlled-substance telemedicine visit

Element	Example wording
Modality and location	"Video visit. Patient located at [address], [state]. I am at [location]."
Identity check	"Identity confirmed by photo ID shown on camera."
Indication and evaluation	Diagnosis and criteria met, functional impairment, and what other treatment has been tried.
Risk review	Substance use history, misuse risk, cardiovascular and other medical risks, suicide risk, and interactions.
PDMP	"PDMP reviewed on [date]. No unexpected findings." Or describe findings and response.
Decision and quantity	Medication, dose, days' supply, and why that supply is clinically appropriate.
Consent	Telemedicine consent and treatment agreement signed on [date].
Monitoring plan	Follow-up date, testing, and when an in-person visit will be required.
Attestation	"Prescription issued for a legitimate medical purpose in the usual course of my professional practice."

Part D: Treatment agreement for controlled medication (patient-facing)

Use for stimulants, benzodiazepines, sedative-hypnotics, opioid use disorder medication and other controlled medications where your practice uses an agreement. Adapt to your specialty and state rules.

Agreement about my medication with [Prescriber name]. This agreement explains how we will use [medication name] safely. It is part of my care and I can ask questions at any time.

1. One prescriber, one pharmacy. I will get this medication only from [Prescriber]. I will use one pharmacy, [pharmacy name and location], unless we agree otherwise in advance. I will tell [Prescriber] about every other medicine, including from other doctors, and about alcohol, cannabis and other drugs.

2. Taking it as prescribed. I will take it only as prescribed. I will not change the dose or share or sell it. I understand that stopping some medicines suddenly can be harmful, and that I should call before stopping.

3. Refills. I will plan ahead. I understand that refills are done at scheduled visits or with [number] business days' notice, and that early refills are not usual.

4. Storage. I will keep this medicine in a locked or secure place away from children, visitors and pets.

5. Lost or stolen medicine. I will report it right away. I understand replacement may not be possible, and that I may be asked to file a report.

6. Monitoring. I agree that [Prescriber] may check the state prescription monitoring database, ask for urine or other tests when appropriate, talk with my other prescribers and pharmacist, and ask me to bring my medicine to a visit for a count.

7. Telehealth conditions. I understand that my visits may be by video, that I must be in [state(s) where prescriber can treat] and tell my prescriber where I am each time, and that the law may require an in-person visit. If the rules change, we will talk about what that means for my care.

8. Safety. I will not drive or use machinery if the medicine affects my alertness. I will tell [Prescriber] if I become pregnant or plan to, and about any thoughts of harming myself.

9. If this agreement is broken. Serious or repeated problems may lead to a change in my treatment plan, a taper or transfer of care. I will not be abandoned. [Prescriber] will help arrange a safe alternative and follow-up.

Patient signature

Date

Prescriber signature

Date

Part E: Planning for a January 1, 2027 return of the in-person rule

Do this in the fourth quarter, whether or not DEA acts. It is much easier to plan for a deadline that does not arrive than to explain one that does.

Step	When	Done
List every patient receiving controlled medication by telemedicine. Note medication, state, and whether the patient has ever been seen in person by you.	By end of October	

Step	When	Done
Check DEA, HHS and your state board for a new extension, a final rule, or state changes.	Monthly, and in the first week of December	
For patients who have never been seen in person: decide who needs an in-person visit, where (your office, a partner clinic or a collaborating DEA-registered practitioner) and when.	By mid-November	
Review your licenses and registrations for the patient's states, and whether you can lawfully see patients in person there.	By mid-November	
Send the patient notice (below), adapted to the situation.	By December 1	
Prepare for transfer: identify colleagues who can accept patients if you cannot lawfully continue. Use your Network and referral tools and make warm handoffs.	By December 15	
Update your treatment agreements and consent forms to reflect any new rule.	When the rule is known	

Patient notice: possible change in telemedicine prescribing

Dear [Patient first name],

The federal rules that allow me to prescribe some controlled medications by video without an in-person visit are currently set to expire on [December 31, 2026]. The government may extend them, replace them or let them lapse. I will let you know as soon as I know more.

To protect your care in every case, I would like to talk with you at your next visit about [scheduling one in-person visit / how we would continue if the rules change]. Please do not stop or change your medication on your own. If you have questions, call [phone].

Sincerely, [Prescriber name]

IN PSYALLIANCE

Psychiatrists in a state can use the Partner and Bench tiers to line up in-person coverage for one another's tele-patients. Post a "tele-prescriber seeking in-person partner" request in the relevant Town Hall channel without identifying any patient, and use the Planner's bulk outreach to line up coverage before a deadline.

Sources and further reading

- DEA and HHS, Fourth Temporary Extension of COVID-19 Telemedicine Flexibilities for Prescription of Controlled Medications, Federal Register, Dec. 31, 2025 (federalregister.gov).
- DEA Diversion Control Division, telemedicine and Special Registration information (deadiversion.usdoj.gov); DEA press release Dec. 31, 2025 (dea.gov).
- HHS, Prescribing controlled substances via telehealth (telehealth.hhs.gov).
- Ryan Haight Online Pharmacy Consumer Protection Act of 2008; 21 CFR 1311 (electronic prescriptions for controlled substances).
- Interstate Medical Licensure Compact (imlcc.org) for eligible physicians seeking multi-state licenses.

ABOUT THIS DOCUMENT

Prepared by PsyAlliance from publicly available professional and regulatory sources, reviewed against them as of September 2026. It is an educational template and practice aid. It is not legal, ethical, clinical or billing advice, and it does not create a professional relationship. Laws, licensing-board rules and payer requirements vary by state and change over time. Verify against your licensing board, professional association, payer contracts and, where the stakes warrant it, a qualified attorney before relying on it.

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